

POLICYREADY • TENNESSEE

How to Open a **Group Home Business** in Tennessee

Family Model & Community Living Supports under ECF CHOICES

Step by Step

*From “where do I start?” to your first payment —
whether you start with a family model home, or your own agency.*

A complete, plain-language guide for real people — not lawyers or consultants.
Figures current for 2026 • Verify all details with the State of Tennessee.

Read this first

This is a plain-language roadmap built from Tennessee's actual rules, agencies, and program manuals. It is not legal, tax, or billing advice, and the details change. One thing that makes Tennessee different from most states: it runs its I/DD services through managed care. You get credentialed by DIDD and then contract with managed care organizations (MCOs) to be paid — there's no billing the state directly. Before you act, confirm the current specifics with DIDD, TennCare, the MCOs, your local fire authority, an attorney, an accountant, and an insurance broker. Every rule and citation here is real, but always check the current version before you rely on it.

What this guide covers

- Part 0 — The big picture: ECF CHOICES, the managed-care model, family model vs. group homes, and the two ways in.
- Part 1 — Path A: a family model residential (host home) provider (the small, live-with start).
- Part 2 — Path B: becoming your own credentialed provider agency (group homes / supported living).
- Part 3 — Who you need on your team.
- Part 4 — The home: what your house needs (the home study, DIDD life-safety inspections, fire safety).
- Part 5 — Getting your first person placed.
- Part 6 — How you get paid: credentialing, MCO contracts, and your first payment.
- Part 7 — What it costs and how long it takes.
- Part 8 — When they say “check locally”: who to call and exactly what to ask (Nashville & Memphis worked out).

PART 0

The Big Picture

Opening a group home in Tennessee is not like opening a normal small business, and the reason people get lost is that the information lives in a dozen different places — and Tennessee's managed-care model adds a twist. Here it is in one place.

The managed-care model — the thing that makes Tennessee different

Most states pay providers directly through Medicaid. Tennessee doesn't. Its I/DD services run through Employment and Community First (ECF) CHOICES, a program administered by TennCare (the state Medicaid agency) and delivered through private managed care organizations (MCOs). To get paid, you do two things: get credentialed by DIDD, then sign a contract with one or more MCOs. Also important: Tennessee's older 1915(c) waivers are closed to new enrollment — all new members come through ECF CHOICES.

Family model vs. group home — what you're actually opening

- Family Model Residential (also called Host Home or Companion Care, and billed as Community Living Supports – Family Model, or CLS-FM) — an adult lives in a private home with a trained caregiver or family, rather than a facility. The small, host-home-style option.
- Community Living Supports (CLS) / Supported Living — a home supporting up to four adults with 24/7 awake staff. The group-home model; larger homes may need a Supported Living or Residential Habilitation license.

The players — learn these names, you'll deal with all of them

- DIDD — the Department of Intellectual and Developmental Disabilities (now within the Department of Disability and Aging). It's the credentialing authority for providers and inspects homes.
- TennCare — Tennessee's Medicaid agency; it runs ECF CHOICES and sets the rates.
- MCOs — the managed care organizations that administer ECF CHOICES and pay you: BlueCare, Amerigroup/Wellpoint, and UnitedHealthcare. You contract with them.
- Support Coordinator — the person (through the MCO) who develops the person-centered support plan and coordinates services.
- DDA Regional Intake Office — your first local point of contact for eligibility and self-referral help.

The fork in the road — there are two ways in

- Path A — Provide family model residential (host home). One adult (sometimes two) lives with you; you complete a DIDD home study and work with a credentialed agency (or become a small one). Fastest, simplest, closest to a host home.
- Path B — Become your own credentialed provider agency. You get credentialed by DIDD, contract with the MCOs, and run CLS/Supported Living homes (up to four residents) — with licensing and life-safety inspections. Much bigger lift, but it's the real business.

PART 1 • PATH A

Family Model Residential (Host Home)

Start here. It's the fastest way in — and you can grow into your own agency later.

Step 1 — Make sure it fits your life

Family model means an adult with a developmental disability lives with you and shares your everyday life. Everyone in the household is part of the process, so this is a family decision — go in with eyes open about the good days and the hard ones.

Step 2 — Connect with a credentialed provider agency

Most family model caregivers work with an established, DIDD-credentialed provider agency that holds the ECF CHOICES contracts, supports you, and handles the billing. Ask your DDA Regional Intake Office for agencies in your area, and interview more than one — how they support and pay family model caregivers varies.

Step 3 — Complete your DIDD home study

Family model (CLS-FM) providers must complete a DIDD home study before anyone is placed. This is the heart of family model approval — it looks at your home, your household, and the match.

Step 4 — Pass background checks & training

Complete the required criminal background check, the DIDD training, CPR/First Aid, and abuse-prevention training. Your agency walks you through the checklist.

Step 5 — Get matched

The agency, the person's Support Coordinator, and the team match you with someone whose needs and personality fit your home. Tennessee builds around person-centered planning and the person's choice — a good match is what makes it work.

Step 6 — Put your policies in writing

You'll keep clear documentation — medications, emergencies, incident reporting, and the rights of the person you support (including their lease/residency protections). PolicyReady's Tennessee manuals are built for exactly this: ready to customize, just add your details.

Step 7 — Get supported, and get paid

The agency bills the MCO and pays you. Because the person lives in your home, your family model payments may qualify for the IRS “difficulty-of-care” exclusion (potentially non-taxable) — confirm with a tax professional. That first payment is the moment it all clicks.

PART 2 • PATH B

Become Your Own Credentialed Provider Agency

More work, more money, more control. Here's the whole thing, in order.

Step 1 — Set up the business

- Form your entity (LLC or corporation) with the Tennessee Secretary of State and get your EIN.
- Get a Type 2 (organizational) NPI (free).
- Line up insurance and, for supported living, the clinical supports the standards require (for higher-need homes, a master's-level clinician and psychiatry access).

Step 2 — Get credentialed by DIDD

DIDD is the credentialing authority for ECF CHOICES (and 1915(c) and Katie Beckett) providers. You submit the credentialing application and the required Settings Rule self-assessment; the Provider Credentialing Committee reviews applications (it meets monthly). This is the gate you must pass before an MCO will contract with you.

Step 3 — Contract with the MCOs

Credentialing lets you serve, but the MCOs pay you — so you contract with one or more of the three ECF CHOICES MCOs (BlueCare, Amerigroup/Wellpoint, UnitedHealthcare). Each has its own contracting and billing system; contract with as many as you want to serve their members.

Step 4 — License & inspect your homes

For supported living or residential habilitation, you'll hold the appropriate DIDD license, and DIDD Housing Inspectors complete a life-safety inspection of each new home before occupancy (and again every 24–30 months). Group/CLS homes are capped at four residents (regardless of funding source). See Part 4.

Step 5 — Policies, staffing & serve

Put your policies and procedures in place (PolicyReady manuals), hire and train your direct support staff (24/7 awake staff for supported living), and make sure each member has the required lease/residency agreement. Then accept referrals, deliver services, and bill the MCOs (Parts 5 and 6).

PART 3

Who You Actually Need (Staffing)

Family model (Path A): you and your household are the support. The credentialed agency provides the professional oversight, the home study, and the billing, and helps arrange respite. You don't build a staff.

CLS / Supported Living agency (Path B), plan for these roles — one person can wear several hats at the start:

- Agency director / administrator — runs the agency, owns credentialing and MCO contracts.
- Direct support professionals (DSPs) — the caregivers in your homes; supported living requires awake staff 24/7.
- Master's-level clinician & psychiatry access — required for homes serving members with significant behavioral/mental-health needs.
- Registered Nurse — for health oversight and medication; often part-time or contracted at first.
- Billing / administrative support — handles MCO billing (EDI, claims within 120 days) and documentation.

PART 4

The Home: What Your House Actually Needs

How Tennessee handles homes

A family model home is approved through a DIDD home study — it's a private residence, not a licensed facility. A supported living or residential habilitation home is inspected by DIDD Housing Inspectors, who complete a life-safety inspection before anyone moves in and then re-inspect every 24–30 months. Those inspections check fire and emergency safety and HCBS-settings compliance (this is someone's home, with lease-like protections).

“Do I need fire sprinklers?” — the honest answer

Family model homes (one, sometimes two people) and CLS/supported living homes (up to four) are small residences, so commercial fire-sprinkler systems generally are not required. What you must pass is the DIDD life-safety inspection, which looks at fire and emergency safety — exits, alarms, and evacuation — for the home. Actual sprinkler and building requirements come from your local fire authority and building code and depend on the home's size and occupancy, so confirm anything unusual (a larger or modified home) with DIDD Housing and your local fire authority before you commit to a property.

THE HOME CHECKLIST

- For family model: a completed DIDD home study of your home and household (and re-checks as required).
- For supported living / residential habilitation: a DIDD Housing life-safety inspection before occupancy, then every 24–30 months.
- Working smoke and carbon-monoxide detectors, a fire extinguisher, and clear exits with a practiced evacuation plan.
- A safe, clean, sanitary home — sound utilities and electrical, working plumbing, safe food storage and preparation.
- A lease or residency agreement for each member that gives them the same legal protections as a lease (an HCBS-settings requirement) — keep a copy for audits.

Two more things people get wrong

- Home capacity is capped. No more than four service recipients may live in a CLS/residential-habilitation home, regardless of program or funding source (three in some blended-home situations). Plan your capacity accordingly.
- Room and board is separate from your pay. The service rate doesn't cover the person's rent and food — those come from their own income (SSI/SSDI). Keep the two separate; many family model caregivers also serve as the member's representative payee.

PART 5

Getting Your First Person Placed

You can't get paid until an MCO member is placed with you and services are authorized. Here's how that happens.

1. The person qualifies. They have an intellectual disability (before age 18, IQ under 70) or a related developmental disability (before age 22), are a Tennessee resident, and have (or are eligible for) TennCare. Eligibility and “benefit group” are set by assessment — the person doesn't pick their group.
2. They enroll in ECF CHOICES. All new enrollment is through ECF CHOICES (the old waivers are closed). Funding is limited, so not everyone who applies can enroll right away.
3. The plan is built. The MCO's Support Coordinator develops the person-centered support plan, which identifies residential supports (family model or CLS/supported living) at the appropriate level.
4. The match and authorization. You (or your agency) are matched to the member based on fit and your credentialed services; the MCO authorizes the service and level.
5. Move-in. With the plan and authorization in place, the person moves in and you're delivering a billable service.

PART 6

How You Get Paid: Credentialing, MCO Contracts & Your First Payment

The two keys that unlock payment

- Credentialing (DIDD): DIDD reviews and approves you as a provider — the state's quality gate. Without it, you can't serve ECF CHOICES members.
- MCO contract: the MCO actually pays the claims. You contract with one or more of BlueCare, Amerigroup/Wellpoint, and UnitedHealthcare, and bill each for its own members.

How the rate works

- Set by TennCare, by level: ECF CHOICES rates for residential supports are established by TennCare and set by the member's assessed level (ECF CLS levels), based on medical-necessity criteria — not by how many people live in the home.
- Family model vs. CLS/supported living: family model (CLS-FM) and CLS/supported living have their own rate structures; the level reflects the person's needs.
- Paid for service actually delivered: you're reimbursed only for dates the member actually receives the service — not days they're in a hospital or on therapeutic leave. The rate is inclusive of most transportation.
- Where to look: exact codes and current rates are in each MCO's provider manual and TennCare's ECF CHOICES materials. Confirm the current figure before you budget.

Your first payment

6. Confirm the MCO's authorization matches the service and level.
7. Deliver the service and keep your daily documentation.
8. Submit the claim to the MCO (electronically / EDI) — within 120 days of the date of service.
9. Get paid — and bill only for days the member actually received the service.

PART 7

What It Costs & How Long It Takes

Timeline

Family model (Path A) can move relatively quickly once you've chosen an agency — the home study, background checks, and training. Becoming your own credentialed agency (Path B) takes longer: the business setup, DIDD credentialing (the committee meets monthly), contracting with each MCO, and licensing and life-safety inspection of each home. Plan for several months end to end, and confirm current timelines with DIDD and the MCOs.

Startup line items

Family model costs are modest — mainly your time, background checks, training, and getting your home ready. A credentialed agency adds business formation, insurance, the clinical supports supported living requires, staff payroll (24/7 awake staff), licensing and inspection, and your policies and procedures. Confirm current fees with DIDD and the MCOs.

PART 8

When They Say “check Locally”: Who to Call & What to Ask

In Tennessee, DIDD and the MCOs are the center of gravity, with your DDA Regional Intake Office and local fire authority close behind. Here's who to call and exactly what to ask.

The statewide offices (same for everyone)

For...	Contact
Becoming a credentialed provider	DIDD (Dept. of Disability & Aging) — tn.gov/disability-and-aging → Become a Credentialed Provider
Getting paid (contract & billing)	The MCOs — BlueCare, Amerigroup/Wellpoint, UnitedHealthcare (800-690-1606)
Which MCO / member help	TennCare MCO line — 1-855-259-0701
Eligibility & self-referral help	DDA Regional Intake Office (West/Middle/East — see below)
Program rules & rates	TennCare ECF CHOICES materials · rule 1200-13-01

The local calls — and what to ask on each one

① DIDD CREDENTIALING / A CREDENTIAL AGENCY — YOUR MOST IMPORTANT CALLS

- “I want to become a family model provider / a credentialed CLS agency. What's the credentialing process, and what's the current provider need in this area?”
- (To an agency) “How do you support family model caregivers, how do you match, and how and when do you pay?”

② DIDD HOUSING — THE LIFE-SAFETY INSPECTION

- “What does the life-safety inspection check for a supported living home, and can I get the inspection tool to self-assess before I open?”

③ LOCAL FIRE AUTHORITY / BUILDING — THE HOME

- “I'll operate a small residential home for up to four adults with disabilities. What fire-safety requirements apply, and is anything needed beyond alarms, extinguishers, and exits?”

Fully worked examples — Tennessee's two biggest metros

Most people start in or near one of these two. Tennessee is divided into three DDA regions — West, Middle, and East — each with an intake office.

NASHVILLE (MIDDLE TENNESSEE)

- DDA Regional Intake Office: Middle Tennessee — (800) 654-4839 (covers the Nashville area).
- Credentialing: DIDD (statewide) — tn.gov/disability-and-aging; then contract with the MCOs.
- Home & fire: Metro Nashville/Davidson County fire authority for fire-safety requirements; DIDD Housing for the life-safety inspection.
- The local quirk here: Nashville is the state's busiest market — build relationships with MCO Support Coordinators, since members choose their provider.

MEMPHIS (WEST TENNESSEE)

- DDA Regional Intake Office: West Tennessee — (866) 372-5709 (covers the Memphis area).
- Credentialing: same statewide DIDD process; then MCO contracts.
- Home & fire: City of Memphis / Shelby County fire authority; DIDD Housing for the life-safety inspection.

- The local quirk here: confirm whether your address is inside the City of Memphis or unincorporated Shelby County — it changes your fire and building contacts.

How to find your own region & offices

10. Call your DDA Regional Intake Office — West (866) 372-5709, Middle (800) 654-4839, or East (888) 531-9876.
11. Start the provider process at tn.gov/disability-and-aging (Become a Credentialed Provider), then contract with the MCOs.
12. For fire safety, call your city or county fire authority; for the life-safety inspection, work with DIDD Housing.

CORE RULES & REFERENCES

- ECF CHOICES service definitions (TennCare) — Community Living Supports (CLS) and CLS – Family Model, and supported living.
- DIDD Credentialing Protocol — the mandatory process for new and expanded provider services (credentialing authority since 2023).
- TennCare rule 1200-13-01 (Long-Term Care Programs) and T.C.A. § 33 — eligibility, benefit groups, and program authority.
- The MCO provider manuals (BlueCare, Amerigroup/Wellpoint, UnitedHealthcare) — CLS/CLS-FM standards, rates, and billing.

You can do this.

Opening a group home in Tennessee is real work — the managed-care system has a couple of extra steps — but it's a real, walkable path, and people across the state do it every day, giving someone a real home in the community. You don't have to be a lawyer or hire a consultant. You need a safe home, a good heart, and a clear map — and now you have the map. Whether you start with a family model home or build your own credentialed agency, this guide takes you from your very first question all the way to your first payment.

Take it one step at a time, verify the current details with the State and the MCOs as you go, and lean on your DDA Regional Intake Office — that's what they're there for.

Prepared for PolicyReady.org. Grounded in Tennessee DIDD and TennCare rules and the ECF CHOICES program materials as of 2026.

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